

I: First question, familiarity with AI. How familiar are you with AI technologies in general?

P: I'm not an expert, but I have used AI in my job to help synthesize reports or come up with lesson plans. It's all about writing good prompts. I know the school system is now using AI more and teaching kids responsible AI use, and we've had some training on that. I also know students who use it quite regularly when they're upset.

I: Is the school teaching staff or also the children?

P: Both.

I: And the use you've used it for, was that something the school approved, or was it your own choice?

P: Actually both. We have Copilot as part of our work software. And the federal government just came out with something called [GOVERNMENT AI TOOL]

I: [GOVERNMENT AI TOOL]?

P: Yes. That's one we have to use, it's only for people who work in the federal government and you need an ID card as a security measure. Copilot is used through the school and through our email, our principal uses it to clean up emails. There's also a site called [AI TOOL], which I joined because it helps interpret and rewrite things and is supposed to make good graphs. It's about \$15 a month and I'm not sure I'll keep it, because sometimes if I upload a PDF with scores it reports them wrong, mixing things up or using the wrong names. Copilot didn't used to do bell curves but now it does, and I've written prompts for that so I can put visuals in my reports, because visuals help parents. I used to use Google Gemini for that but they stopped it unless you pay more. So I've been trying to fine-tune what works for me.

I: So there have been recommendations from both the school and the government to use specific tools. Did you disclose this use with any of your clients?

P: I've put it in my report that some of it has been used. There's a standard phrase saying some of it was assisted by AI and that I'm responsible for editing.

I: On a scale of one to ten, how familiar do you think you are with AI?

P: About a five or six.

I: Then the next question is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think it has a role. When I've done assessments I can put scores in and ask it to make graphs. For example, I had a rating scale with a parent, two teachers, and a student, and I asked it to make a graph showing how their scores compare. I haven't asked it to analyze or interpret the scores, I do that myself. But once I've written my report, which can be 30 pages, I

have to summarize it for another system, and I've asked AI to synthesize what I've written and it does a good job. I proof it, but that works. I wouldn't trust it to interpret test results on its own.

I: Do you have any intake processes, or how does your day-to-day work look?

P: I work in a school, so it's not like I need parent permission to work with a student. I introduce myself to the student and I generally know who they are because they're in my school. That's also why I'm not using AI with them in the room. I've been to doctor's appointments where they ask if they can record to synthesize and write up notes, but I don't do that. I would need parent permission to record a student, so I don't. I don't know if other people do, but I don't.

I: Do you think that would be a possible benefit, or would you want to use that?

P: I don't know that I would. There's confidentiality in meeting with a client or a student. If you're recording it there's a record somewhere that can be subpoenaed for court. I tend to be careful about what I put in writing. I'll document that I saw a student and put a broad subject of what we talked about, but I'm not writing clinical notes. I can see where, if you're an adult and you consent in a therapy session, that might be different, but I don't use it that way.

I: So it can be subpoenaed, regarding you, the client, or both?

P: Both. When you work in a school you work for a school district. Some of the students I see are special education, and there's documentation of how many minutes I have to see them and what goals I'm working on. Any of those documents can be subpoenaed if a parent feels I'm not providing the correct service. For example, I could be working with a student on IEP goals and they might tell me they're gay or bi or something else. If I had that written down it could also be subpoenaed. So I write what I need to document their progress on the goals. Or if I'm working with a student who's suicidal and they kill themselves, the parents could subpoena all of my documents.

I: How do clients come to you? Does the child decide they want to come, or is it the parent?

P: A little bit of both. In high school, ages 14 to 18, students will often come and request to see me. I can see them once on a topic without parent permission, for example if they broke up with a partner, and again on a different topic like bullying. But if it's the same topic twice I have to get parent permission. For elementary students, sometimes they come and ask what I do and if they can talk to me, and often it's part of their program because they're special education and need support. It's about building rapport and explaining why they're there, then we work on what we need to work on.

I: Then the next question is about monitoring and progress tracking. How do you feel about the role of AI in tracking patient progress over time?

P: I think it's really helpful. You can put in your prompt what you want documented or monitored and have it produce a graph or summary. I have to write progress reports every quarter for students on an IEP, and I can put in the goal, type in observations over time, and ask it to write

something that includes all of that. I just did an observation on a student for an hour and a half and was able to verbally speak my observations with timestamps into Copilot. It did a time study of how much time they were on task and also wrote up a narrative of the assessment, in much less time than doing it myself. It could be a really useful tool if you know the prompts you need. You also have to be sensitive to confidentiality. I don't put the student's name in at all. I put it in my own way so I can search and replace it later.

I: What is an IEP?

P: Individualized Education Plan. If a student is receiving special services because they have a learning disability or ADHD or similar, that's what we call the services we give them, individualized to them.

I: The Copilot use, how did that work during the observation?

P: On our system, because we use Teams and Microsoft, if you press Windows and H at the same time a microphone comes up. When Copilot is open you just speak into it and tell it your prompt. I was able to say, this is an observation, could you write it up with timestamps, put it in narrative form, do a task analysis of time on task based on the timestamps, and graph it. I went into the classroom and wrote all the information down with timestamps, then when I came back to my office I turned on the microphone and read it out.

I: And for prompting, what AI tool do you usually use?

P: Right now Copilot pretty much exclusively. Google Gemini stopped making the graphs I wanted.

I: How would you feel about AI taking on monitoring tasks in your practice?

P: I think it has a good role if it's used well and ethically. You have to be careful about privacy and confidentiality, who has access, and things like that. In Copilot, conversations are saved, so who else can access them matters. Everything has to be double or triple checked because it's not always accurate.

I: So the main concerns are data privacy and correctness.

P: Yep.

I: Then the next question is about administrative and practical support. How do you feel about AI generating drafts or reports or assisting with scheduling and reminders? You briefly mentioned you already use it for reports and graphs.

P: I think it does a good job with that, but it takes time to develop the prompts to get the information you need. If you don't give it the right prompts you're not going to get good information. What I like about Copilot is that it gives suggestions of what else it could do, like offering to reformat something or change the writing style. If I'm asking it to summarize a report I

can say, as written by a school psychologist for [ORGANIZATION], in this format, please use these headings. With the bell curves now, Copilot remembers my preferred parameters so I don't have to retype everything every time. Before, I was printing out bell curves and drawing the lines myself. Now I can have them color-coded with percentages and standard deviation bands, and it takes less than two minutes.

I: How much time do you think you're saving by using these tools?

P: A couple of hours every week, and probably more if I get better at it. Using it to make graphs inside reports saves a huge amount of time.

I: Did you get guidance from your employer on prompting, or was it mostly trial and error?

P: A bit of both. Some of it is trial and error, some came from the [AI TOOL] program which had built-in prompts, and we also have training every quarter with AI prompt courses on how to structure prompts and what information to include.

I: What concerns would you have about accuracy or liability?

P: I think people could get lazy and rely on it too much without making sure the information or interpretation is correct. Every report I write is a legal document and has to stand up in court, so it matters where the information comes from. Everything is written very carefully, citing parent reported, student reported, progress reports showed. You have to make sure all those steps are still there, and you have to know where the information is going and consider privacy.

I: Who's at fault if something AI-generated is wrong?

P: It would be my fault regardless. I'm the one responsible for making sure everything is correct.

I: Then the next question is about psychoeducation and between-session tools. What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think there's a lot of good use for it. I haven't explored that as much because I focus more on assessment, but depending on the therapy approach you could ask it to create a CBT journal tailored to an elementary or secondary student, or make lesson plans or materials for students to refer to. Our special education teachers have used it to make desk reminders, like steps to calm down or where to go for support. There's a lot of potential, but it depends on how you use it and whether you know the prompts.

I: How does psychoeducation or between-session support usually look now?

P: For example, with a high school student who gets frustrated and pulls her hair out, we talk about coping skills, discuss other ways she can cope, and practice those skills. We often write down steps together during the session and turn that into a chart she can keep in her binder or on her phone. We also check in between sessions and communicate with staff, and monitoring comes from other sources too because students aren't always reliable reporters.

I: Do they also get homework or tasks between sessions?

P: Yes, some do. They might document how often they felt a certain way and what coping strategy they used. We practice coping techniques in session and then they try them out, documenting what they did, how long it took, and what worked best. It depends on the student and their needs.

I: What if the AI provides information that contradicts your treatment approach?

P: I think everybody has their own biases, so you have to look at whether it's your own bias, what the better information is, and review it to determine what's best for the client. AI can produce a lot of things but it doesn't know the client as well as you do. It's really the clinician's interpretation of what's best. One of the downfalls of AI is that it doesn't know the person, so you have to take it with a grain of salt and check it, but also make sure you're not so entrenched in your own view that you're unwilling to look at other options.

I: Would you worry about clients relying too much on AI between sessions?

P: AI isn't a person, so it doesn't have a relationship with you. I think students could get really connected to a bot, and there's so much interaction that isn't real life or with real people. Students are unlearning how to interact with others in person. You say things online or to a bot that you wouldn't say to a real person, and they give generalized replies that are logical but not necessarily about feelings. If you asked AI how to kill yourself and what you have in your house, it might tell you what to do. It wouldn't ask why you're feeling that way, or say that this might be temporary, or ask what other help you can get. I can see people using it because it's more comfortable than talking to a real person, and that comfort could help some people. But I don't think it knows when someone actually needs a real clinician or medication. It's biased based on what the person tells it and their perception, and it doesn't reason about the other side of the story.

I: How would you monitor what the AI is telling your clients?

P: I ask them if they use it and what kinds of responses they're getting. Sometimes there's a record on their phone so we can review that. I honestly haven't had many students I work with regularly who use it, but there's one I'm meeting with soon who has, and I'm curious to see what it says. It comes down to perception and bias.

I: Then the next question is about risks and limitations. What risks or concerns do you have about using AI in clinical work?

P: I think there's a lot of fear that AI will take over human jobs. If a bot can do therapy cheaply or for free, is that the direction things will go? My concern is that, like in the UK, anyone can call themselves a counselor without necessarily needing a degree. So if people are using AI prompts without being trained or certified, there's a risk of grooming or bad information. Who's programming the AI? Who's behind it? Is it being used for good or for bad? And who has access

to the information? Nothing is ever really gone from the internet. Could things people say to AI be used against them later? So again, what happens to the information and where does it go?

I: In the US, being a counselor or psychologist is a protected title, right?

P: Yes, you need specific degrees and training. In the UK anyone can say they're a counselor. There's also a lot of telehealth now, and many students don't get services at [LOCATION] so they use telehealth outside the [LOCATION]. The clinician might not see things like skin picking or fidgeting under the table, and a lot of nonverbal communication is missed. That face-to-face interaction is important, and I worry it's already going away because of social media.

I: Have you thought about issues like bias?

P: Whoever is programming AI matters, because AI learns but it's also shaped by whoever programmed it. When I was in grad school we had to think a lot about our own biases. For example, if I'm a born-again Christian and don't believe in abortion or birth control, and a young woman comes in with issues related to that, am I the appropriate person to give a non-biased view? Or do I need to recuse myself? AI isn't going to do that. You ask a question and it gives you an answer, but I don't know who programmed it or what philosophy they're using. And it won't necessarily ask all the questions a therapist would to really know the client or show empathy.

I: What concerns do you have about transparency or explainability of AI decisions?

P: There have been cases of students or teens talking about suicide and AI giving advice about ways to kill yourself. It's bad enough that kids who cut or are anorexic can go online and find sites that tell them how to hide it or how to burn calories or cut so nobody sees. So does AI just provide a bigger resource for people to use in ways that aren't helpful or therapeutic? And then there's the issue of incomplete information. When I work with a student I get information from teachers and parents, so I have a fuller picture, not just the student's report. AI wouldn't have that. A student might not be an accurate reporter of what's going on, and without outside information it's really hard to treat them properly.

I: Any other limitations you can think of?

P: Those are what I can think of now.

I: Then the next question is about client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment?

P: I think it depends on how you explain it and what parts you're using. I wouldn't have them sit there and talk to Copilot about their problems. I'm not doing it in front of a client and I'm not recording them. It's more like, here are some exercises or handouts you can use. I could have made them myself but it would have taken hours. I'm not giving them anything I wouldn't have researched and proofed myself. People can be lazy and not proof things, and then misinformation can happen. Prompts themselves can also be biased based on your own biases.

I: How willing do you think clients are for AI to be used in their treatment, on a scale of one to ten?

P: It depends on the client. Teenagers and younger people under 25 or 30 would probably be around a seven or eight. People over 40 or 50 would be much lower, maybe a three. They're not as comfortable or knowledgeable about how it works. And it depends on how it's being used. I'm not recording sessions and letting Copilot generate what I say to clients. I'm still using my own skills and knowledge, so it depends on how much you're relying on it.

I: How do you think the therapeutic alliance would be affected if you used AI during meetings or in general?

P: I think clients would wonder if you're really listening to them or relying on somebody else to listen and come up with things. The whole point is to sit face to face, have a conversation, build rapport, and know when to ask probing questions or when not to. Knowing when the person is ready, when to push or not, and what questions to ask. I don't think AI could do that as well without knowing the person, and I don't think it would improve trust. The client might think they can just do this on their own, so why are they seeing you? In a clinical setting where someone is paying for therapy, they might think, why am I paying you when I could just do this myself? That trust and rapport with the therapist really matter.

I: Do you think clients would be more or less engaged with AI-supported treatment?

P: It depends on the client. Some may prefer it. I have a student who has been using some AI tools and is worried about sharing things with their therapist because they're afraid of being judged. They read into body language and facial expressions and think someone is disappointed or upset. Some clients might prefer AI because you don't see facial expressions. But others are fine with face-to-face and supplementing it with resources. I often give parents or students handouts or steps to calm down, and now I can make those faster with Copilot, but it's the same information. Clients would be more upset if I relied on Copilot to actually do the therapy or decide what to talk about.

I: For some people who are uncomfortable sharing nonverbally, AI might feel safer. Do you think it's better to use AI in those cases, or is face-to-face still better?

P: I think building the relationship and rapport is still better. Eventually clients can share things they're uncomfortable saying at first. You can also use journaling, texting, or writing things down and sharing them. There are ways to handle that without AI. I think people could get too comfortable with AI and lose the ability to relate to others, and I still think having a person involved is better than relying on AI alone.

I: For the last question, this is about professional identity, autonomy, and future conditions. Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

P: The actual one-on-one counseling or therapy session should remain human. You can supplement with handouts, homework, or resources, and AI can help combine things into a clear handout. But picking up the phone and talking to a parent is much better than sending an email or an AI-scripted message. Hearing someone's voice, knowing if they're upset, and asking follow-up questions matter. Having connections with teachers, parents, coaches, and others gives a well-rounded view of the student that you can't get through email. AI is similar in that way, you can't hear tone.

I: Do you have anything else to add?

P: I think it will be interesting to see what your research shows. AI and mental health are a big topic right now and a lot of people are relying on it.